

Women’s Health Requirements Discovery Pack

Gynaecology Detailed Rule Review

These are working notes for the gynaecology grading rules. Please mark anything that differs from local practice.

Prepared for requirement discussion.

Service	Rule ID	Category or scenario	Source document	Our understanding of the rule	What information does the grader need to make this decision?	Expected outcome or priority	Agree as written? (Yes / No / Amend / Not used locally)	If not please amend	Needed in phase 1? (Yes / No / Later)	Clinical owner	Notes
GYNAECOL OGY	GYN-01	AUB: no pelvic ultrasound within 12 months	Gynaecology guideline PDF	If there is no pelvic ultrasound within 12 months, the referral appears to be rejected and returned to GP requesting re-referral with scan.	Referral content, ultrasound availability	Reject -- request re-referral with ultrasound			Yes		
GYNAECOL OGY	GYN-02	AUB: endometrial thickness >15mm or no recent endometrial sample, with or without adenomyosis	Gynaecology guideline PDF	If ultrasound shows endometrial thickness >15mm and there is no pipelle or endometrial sample in the last 6 months, with or without adenomyosis, this appears to be P2 Menorrhagia.	Ultrasound findings, endometrial thickness, biopsy history	P2 Menorrhagia			Yes		
GYNAECOL OGY	GYN-03	AUB: ultrasound suggestive of polyp	Gynaecology guideline PDF	If ultrasound is suggestive of polyp, with or without dysmenorrhoea, this appears to be P3 Hysteroscopy.	Ultrasound findings, symptoms	P3 Hysteroscopy			Yes		
GYNAECOL OGY	GYN-04	AUB: ET <15mm, otherwise normal, but bleeding >3 months and medical management already tried	Gynaecology guideline PDF	If ET <15mm and otherwise normal, but intermenstrual or continuous bleeding has lasted >3 months and medical management has been tried, this appears to be P3 Menorrhagia. If not tried, the note appears to suggest medical treatment first.	Ultrasound, duration, medications already tried	P3 Menorrhagia or medical management first			Yes		
GYNAECOL OGY	GYN-05	AUB: post-coital bleeding with normal smear in premenopausal patient	Gynaecology guideline PDF	Post-coital bleeding with normal smear in a premenopausal patient appears not to need specialist review.	Smear result, menopausal status	Does not need to be seen			Yes		
GYNAECOL OGY	GYN-06	AUB: fibroid <3cm found incidentally on ultrasound	Gynaecology guideline PDF	If ultrasound shows fibroid <3cm, no other abnormalities, and normal ET, this appears to be declined with advice that the fibroid is incidental. If medical management has already been tried, it appears to become P3 Gynae.	Ultrasound, fibroid size, endometrial thickness, prior management	Decline or P3 depending on prior management			Yes		
GYNAECOL OGY	GYN-07	AUB: abnormal cervix with normal smear	Gynaecology guideline PDF	If there is an abnormal cervix with normal smear, this appears to be P2 Gynae and requires SMO grading.	Cervical appearance, smear result	P2 Gynae (SMO grading)			Yes		
GYNAECOL OGY	GYN-08	PMB: no ultrasound within 6 months	Gynaecology guideline PDF	If there is no ultrasound within 6 months, the referral appears to be rejected and sent back requesting ultrasound. The note suggests POAC if cost is a barrier.	Ultrasound availability, history	Reject -- request ultrasound			Yes		
GYNAECOL OGY	GYN-09	PMB: single episode with ET <4mm	Gynaecology guideline PDF	Single episode PMB with ET <4mm appears to suggest community pipelle if appropriate, ovestin cream, and re-referral only if PMB recurs.	PMB episodes, ultrasound, ET	Community management / re-refer if recurrent			Yes		
GYNAECOL OGY	GYN-10	PMB: one or more episodes with ET >=5mm	Gynaecology guideline PDF	One or more episodes of PMB with ET >=5mm appears to be HSC P1 and Rapid Access Clinic hysteroscopy.	PMB episodes, ultrasound, ET	HSC P1 / Rapid Access Clinic			Yes		
GYNAECOL OGY	GYN-11	PMB: multiple episodes with ET <5mm	Gynaecology guideline PDF	Multiple episodes of PMB with ET <5mm also appear to be HSC P1 / Rapid Access Clinic.	PMB episodes, ultrasound, ET	HSC P1 / Rapid Access Clinic			Yes		
GYNAECOL OGY	GYN-12	PMB: other ultrasound abnormalities	Gynaecology guideline PDF	If ultrasound shows other abnormalities, the note appears to require SMO-only grading and clinical judgement.	Ultrasound findings	SMO clinical judgement			Yes		
GYNAECOL OGY	GYN-13	Fibroids >3cm with mass symptoms or hydronephrosis	Gynaecology guideline PDF	Fibroids >3cm with mass symptoms, and/or hydronephrosis, appear to be P2 Gynae.	Fibroid size, symptoms, ultrasound findings	P2 Gynae			Yes		
GYNAECOL OGY	GYN-14	Fibroids <3cm with mass symptoms	Gynaecology guideline PDF	Fibroids <3cm with mass symptoms appear to be P3 Gynae, with a note to GP to explore other causes as well.	Fibroid size, symptoms	P3 Gynae			Yes		

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GYNAECOL OGY	GYN-15	Fibroids <3cm with no mass symptoms or AUB	Gynaecology guideline PDF	Fibroids <3cm with no mass symptoms or AUB appear to be declined and do not need follow-up scan.	Fibroid size, symptoms, bleeding history	Decline			Yes		
GYNAECOL OGY	GYN-16	Ovarian masses or cysts: probable benign cyst	Gynaecology guideline PDF	Probable benign ovarian cysts, including simple cyst or likely dermoid, appear to be Gynae P2.	Mass features, imaging	P2 Gynae			Yes		
GYNAECOL OGY	GYN-17	Ovarian masses or cysts: suspicious complex adnexal mass	Gynaecology guideline PDF	Complex adnexal masses that appear suspicious seem to require SMO-only grading, CT if age >40, tumour markers for all ages, and HSC P1 if clinically appropriate.	Mass features, imaging, tumour markers, age	SMO review / possible HSC P1			Yes		
GYNAECOL OGY	GYN-18	HSC P2 semi-urgent category	Gynaecology guideline PDF	HSC P2 appears to be an SMO-only category for concerning referrals that do not meet HSC P1 but should not wait the standard P2 timeframe. The guideline examples include borderline ovarian cyst, recurrent PMB with normal histology, and premenopausal referral with markedly increased ET.	Clinical judgement, imaging, histology	P2-HSC (SMO only)			Yes		
GYNAECOL OGY	GYN-19	Pelvic pain: no ultrasound	Gynaecology guideline PDF	No ultrasound appears to lead to rejection unless there is already a strong endometriosis history or other exception.	Ultrasound availability, history	Reject unless exception applies			Yes		
GYNAECOL OGY	GYN-20	Pelvic pain: >3 months with normal ultrasound	Gynaecology guideline PDF	Pelvic pain >3 months with normal ultrasound appears to be P3.	Pain duration, ultrasound	P3			Yes		
GYNAECOL OGY	GYN-21	Pelvic pain: endometrioma >=5cm or DIE on imaging	Gynaecology guideline PDF	Abnormal ultrasound with endometrioma >=5cm or DIE on imaging appears to be P2.	Imaging findings	P2			Yes		
GYNAECOL OGY	GYN-22	Pelvic pain: chronic pelvic pain fully investigated and gynae cause treated	Gynaecology guideline PDF	If chronic pelvic pain has been fully investigated and the gynaecological cause has been treated, the note appears to decline and refer to Chronic Pain Service.	Investigation history, treatment history	Decline / refer to Chronic Pain Service			Yes		
GYNAECOL OGY	GYN-23	Urogynaecology: mesh problems	Gynaecology guideline PDF	Mesh-related problems appear to be P2 and SMO-reviewed.	Mesh history, symptoms	P2 (SMO)			Yes		
GYNAECOL OGY	GYN-24	Urogynaecology: procidentia	Gynaecology guideline PDF	Procidentia appears to be P2.	Examination findings	P2			Yes		
GYNAECOL OGY	GYN-25	Urogynaecology: prolapse without procidentia	Gynaecology guideline PDF	Prolapse without procidentia appears to be P3.	Examination findings	P3			Yes		
GYNAECOL OGY	GYN-26	Urogynaecology: prolapse with urinary retention	Gynaecology guideline PDF	Prolapse with urinary retention appears to be P2.	Retention, examination findings	P2			Yes		
GYNAECOL OGY	GYN-27	Urogynaecology: stress incontinence	Gynaecology guideline PDF	Stress urinary incontinence appears to be P3.	Symptoms, prior management	P3			Yes		
GYNAECOL OGY	GYN-28	Urogynaecology: urge incontinence without prior medication trial	Gynaecology guideline PDF	Urge incontinence appears to be rejected unless medications have already been tried.	Symptoms, prior medication trial	Reject unless prior treatment completed			Yes		
GYNAECOL OGY	GYN-29	Urogynaecology: asymptomatic prolapse	Gynaecology guideline PDF	Asymptomatic prolapse appears to be rejected.	Symptoms, examination findings	Reject			Yes		
GYNAECOL OGY	GYN-30	Urogynaecology: previous TVT or sling issue	Gynaecology guideline PDF	Previous TVT or sling issue appears to be P2 and SMO-reviewed.	Previous procedure, symptoms	P2 (SMO)			Yes		
GYNAECOL OGY	GYN-31	Fertility	Gynaecology guideline PDF	Fertility referrals appear to be declined and redirected to Northern Region Fertility Service.	Referral reason	Decline / redirect			Yes		
GYNAECOL OGY	GYN-32	PCOS	Gynaecology guideline PDF	PCOS appears to be managed virtually and not seen face-to-face unless diagnosis is unclear or fertility issues require SMO input.	Symptoms, fertility concern, diagnostic clarity	Virtual / redirect / clinician advice			Yes		
GYNAECOL OGY	GYN-33	Cervical polyp: asymptomatic and <=2cm with normal smear	Gynaecology guideline PDF	Asymptomatic cervical polyp <=2cm with normal smear appears to be rejected with GP monitoring.	Polyp size, symptoms, smear result	Reject			Yes		

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GYNAECOL OGY	GYN-34	Cervical polyp: >=2cm with normal smear and symptoms	Gynaecology guideline PDF	Cervical polyp >=2cm with normal smear and symptoms such as post-coital or intermenstrual bleeding appears to be P3 Gynaecology.	Polyp size, symptoms, smear result	P3 Gynaecology			Yes		
GYNAECOL OGY	GYN-35	Paediatric gynaecology and structural anomalies	Gynaecology guideline PDF	These referrals appear to need SMO grading and may need transfer to paediatric or female multi-disciplinary services depending on the scenario.	Age, presenting issue, imaging, tumour markers	SMO grading / possible transfer			Yes		
GYNAECOL OGY	GYN-36	Pelvic tear	Gynaecology guideline PDF	3A tear appears to be referred to Women's Health physio. 3B, 3C, and 4th degree tears appear to be P3.	Tear grade, symptoms	Physio or P3 depending on grade			Yes		
GYNAECOL OGY	GYN-37	Tubal ligation counselling	Gynaecology guideline PDF	Tubal ligation counselling (MSC) appears to be P3.	Referral reason	P3			Yes		
GYNAECOL OGY	GYN-38	Re-grading patients already on the waitlist	Gynaecology guideline PDF	Patients already on the waitlist who are re-referred with updated information appear to require re-grading and booking-team notification if the priority changes.	Prior grading, new information	Re-grade and update waitlist if needed			Yes		